SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab + carboplatin + fluorouracil

First-line treatment of metastatic or unresectable recurrent head and neck squamous cell carcinoma in adults with tumour PD-L1 CPS ≥1. · NCCP 00705 · version 2d · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pembrolizumab and chemotherapy aim to control head and neck cancer that has spread or returned and cannot be removed. Your team explains the expected benefit, uncertainty and alternatives for you.

Schedule
Every 21 days: pembrolizumab and carboplatin on Day 1; fluorouracil over Days 1–4. Up to 6 combination cycles, then pembrolizumab alone while appropriate. Every dose depends on safety and benefit review.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver or kidneys
- ☐ Thyroid, pituitary, adrenal or glucose problems
- ☐ Severe skin reactions
- ☐ Rare heart, muscle, nerve and eye inflammation
- ☐ Infusion reactions
- ☐ Some immune effects need long-term treatment

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Low platelets and bleeding
- ☐ Anaemia and fatigue
- ☐ Nausea and vomiting
- ☐ Infusion or allergic reactions
- ☐ Kidney, nerve or hearing effects

Fluorouracil
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Mouth and throat soreness
- ☐ Diarrhoea and dehydration
- ☐ Painful palms or soles
- ☐ Heart problems including chest pain
- ☐ Rare severe neurological or early excessive toxicity

Discuss the individual aim, alternatives and uncertainty; serious or lasting harms; treatment burden; fertility and pregnancy precautions; and the patient's priorities. Explain low blood counts/infection, bleeding, nausea, mucositis, diarrhoea, fluorouracil cardiac and early severe toxicity, platinum kidney/hearing/nerve effects, DPD testing limits, pump/line complications and immune organ inflammation including permanent hormone replacement. Separate the up-to-six-cycle combination phase from ongoing pembrolizumab maintenance. Confirm actual urgent contacts and teach-back; this draft does not authorise treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00705 v2d.

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